

Impact Statement Card

Values & Ethical Framework

Which values should guide your solution?

Fairness, Privacy, Human Dignity, Transparency, and Inclusivity.

* Fairness: The model must perform equitably across demographic groups (gender, age, socioeconomic status, regional identity, language, educational background, cultural background) to avoid perpetuating health disparities or marginalizing already vulnerable populations.

* Privacy: Mental health data is among the most sensitive personal information; the system must employ differential privacy techniques, secure data handling protocols, clear informed consent mechanisms, and user control over data retention.

* Human Dignity: The solution must complement rather than replace human clinicians and counselors; it must avoid pathologizing cultural expressions of emotion, spiritual practices, grief responses, or normal stress; it must preserve user autonomy and agency.

* Transparency: Users, families, and healthcare providers must understand how and why the model identifies risk; the system must communicate uncertainty, limitations, and its role as a screening tool, not a diagnosis.

* Inclusivity: The system must be designed for and with marginalized voices (rural populations, low-literacy users, persons with disabilities, non-binary individuals) ensuring the tool serves those most underserved, not just those easiest to reach.

Why did you choose these values?

Mental health work operates at the intersection of medical science, cultural context, spirituality, personal identity, and lived experience.

* Fairness is essential because health AI has historically replicated and amplified existing disparities; without explicit attention to equity, a model trained on urban data will fail rural users, potentially creating a two-tier system where well-resourced populations benefit and vulnerable populations are harmed or excluded.

* Privacy protects individuals from stigma, discrimination, blackmail, and state surveillance (critical in contexts where mental illness carries deep social cost).

- * Human Dignity acknowledges that mental health is not a technical problem to be "solved" but a deeply human condition requiring empathy, cultural awareness, and respect for diverse ways of being and healing.
- * Transparency enables accountability; it prevents the tool from becoming an opaque "black box" that strips agency from users and clinicians, and it builds justified trust.
- * Inclusivity ensures we don't replicate the historic failures of health systems that serve only the privileged; if this tool is to reduce health disparities, it must be designed from the start for those most marginalized.

Ethical Risk Assessment

What ethical risks should your team consider?

- * Misclassification & Harm: False positives may label individuals as mentally unwell when experiencing normal stress or cultural grief responses, causing unnecessary stigma, panic, or inappropriate medical intervention. False negatives miss individuals truly at risk, delaying lifesaving intervention.
- * Data Bias & Inequity: Training data skewed toward urban, educated, English-speaking populations with formal healthcare access will fail on rural communication patterns, informal languages, code-switching, and marginalized voices. The model may perform poorly for women, LGBTQ+ individuals, or ethnic minorities whose expressions of distress differ from training data.
- * Privacy Breaches & Surveillance: Unauthorized access to sensitive mental health disclosures could enable discrimination in employment, insurance, education, or social services. Data could be weaponized by abusive partners, family members, or state actors. Inadequate consent mechanisms may allow data to be used for purposes users did not intend.
- * Over-Medicalization & Pathologization: The model might pathologize grief, cultural expression, spiritual crisis, normal stress responses to hardship, or political anger (imposing a Western biomedical framework on diverse ways of experiencing and healing).
- * Dependency & Professional Deskilling: Over-reliance on AI might erode clinicians' diagnostic skills, intuition, and ability to engage deeply with patients. Community health workers might defer to the model rather than trusting their own judgment and relationships.
- * Vulnerability Exploitation: Bad actors could reverse-engineer the model's architecture to manipulate individuals in crisis, extract confessions, or identify vulnerable populations for exploitation or radicalization.

* Digital Divide & Access: If the tool requires internet, smartphones, or literacy, it may exclude the poorest and most isolated populations (defeating the equity goal).

* Inadequate Referral Infrastructure: Flagging at-risk individuals is only useful if they can actually access counseling or psychiatric care. Poor referral linkages could create harm without remedy.